

1. Administrative & Study Identification

Full Study Title: A Study Evaluating the Safety, Pharmacokinetics, and Efficacy of Mosunetuzumab + Lenalidomide (+Len), and the Safety, Tolerability, and Pharmacokinetics of SC Versus IV Mosunetuzumab + Len in Participants With Follicular Lymphoma **Short Title/Acronym:** C041942 **Protocol Number:** C041942 **NCT Number:** NCT04246086 **Sponsor Name:** Roche (Hoffmann-La Roche) **Investigational Product:** Mosunetuzumab (Subcutaneous [SC] and Intravenous [IV]) + Lenalidomide **Phase of Development:** Phase 1/Phase 2 (Phase Ib/II) **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, pharmacokinetics, and efficacy (complete response/complete metabolic response rate) of mosunetuzumab in combination with lenalidomide in participants with follicular lymphoma (FL), and to compare the subcutaneous (SC) versus intravenous (IV) formulations of mosunetuzumab. **Secondary Objectives:** To evaluate the Objective Response Rate (ORR), Duration of Response (DOR), Progression-Free Survival (PFS), Overall Survival (OS), and immunogenicity profile.

2.2 Background & Rationale Follicular lymphoma is the most common form of slow-growing non-Hodgkin lymphoma. Although front-line treatments exist, patients frequently relapse and the disease becomes increasingly refractory. Mosunetuzumab is an off-the-shelf CD20xCD3 T-cell engaging bispecific antibody that redirects T cells to eliminate malignant B cells. Lenalidomide is a potent immunomodulatory agent that has synergistic effects with anti-CD20 therapy. Combining mosunetuzumab with lenalidomide aims to provide a highly effective, chemotherapy-free regimen with a manageable safety profile for patients with previously untreated or relapsed/refractory (R/R) FL.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Trial Status:** Recruiting **Control Type:** Active Comparator (SC vs IV formulations) **Blinding:** Open-label **Randomization:** Features a non-randomized stage followed by a randomized stage comparing SC versus IV mosunetuzumab.

3.2 Planned Enrollment & Duration Target \$\$\$: Approximately 237 participants **Number of Sites:** Multicenter global study **Estimated Study Start:** 12-Aug-2020 **Estimated Primary Completion:** 06-Sep-2030

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age $\geq$ 18 years. 2. Eastern Cooperative Oncology Group (ECOG) Performance Status of 0, 1, or 2. 3. Relapsed or Refractory (R/R) FL after treatment with at least one prior systemic lymphoma therapy (including prior immunotherapy or chemoimmunotherapy), OR previously untreated participants with FL requiring systemic therapy assessed by investigator based on the Groupe d'Etude des Lymphomes Folliculaires (GELF) criteria and a FLIPI score of 2-5. 4. Histologically documented FL of Grade 1, 2, or 3a expressing CD20 at diagnosis as determined by the local laboratory. 5. Fluorodeoxyglucose avid lymphoma (i.e., PET positive lymphoma). 6. At least one bi-dimensionally measurable nodal lesion (>1.5 cm) or extranodal lesion (>1.0 cm) by PET-CT scan. 7. Availability of a representative tumor specimen and pathology report for FL diagnosis confirmation. 8. Adequate hematologic function and normal laboratory values (unless due to underlying lymphoma). 9. Negative HIV test at screening (Exception: stable on antiretroviral therapy for at least 4 weeks, CD4 count $\geq$ 200/mL, undetectable viral load, and no opportunistic infection attributable to AIDS within 12 months). 10. Agreement to comply with lenalidomide risk minimization plan and use adequate contraception methods (including refraining from donating eggs/sperm).

4.2 Exclusion Criteria 1. Any history of Grade 3b FL, suspicion/evidence of transformed lymphoma, or diffuse large B-cell lymphoma (DLBCL). 2. Active or history of central nervous system (CNS) lymphoma or leptomeningeal infiltration. 3. Documented refractoriness to lenalidomide (no PR or CR within 6 months of therapy) or an obinutuzumab monotherapy-containing regimen. 4. History of idiopathic pulmonary fibrosis, organizing pneumonia (e.g., bronchiolitis obliterans), drug-induced pneumonitis, or active pneumonitis. 5. Treatment with systemic immunosuppressive medications (e.g., prednisone, azathioprine, methotrexate, thalidomide) within 2 weeks prior to Cycle 1 Day 1. 6. History of solid organ transplantation or prior allogeneic hematopoietic stem cell transplant (HSCT). 7. Severe allergic/anaphylactic reaction to humanized, chimeric, or murine MABs; sensitivity to Chinese hamster ovary cells, mannitol, or study drug components. 8. History of erythema multiforme, Grade ≥ 3 rash, or blistering following prior immunomodulatory derivatives. 9. Known active bacterial, viral, fungal infection; active Hep B/C; chronic active Epstein-Barr virus; history of macrophage activating syndrome (MAS) or hemophagocytic lymphohistiocytosis (HLH). 10. Contraindication to treatment for thromboembolism prophylaxis or

Grade 2 neuropathy. 11. Evidence of significant uncontrolled cardiovascular/pulmonary disease, or clinically significant liver disease (viral hepatitis, current alcohol abuse, cirrhosis). 12. Major surgical procedure within 28 days prior to Cycle 1 Day 1. 13. Pregnant or lactating women, or life expectancy < 3 months.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Mosunetuzumab is administered via a step-up dosing regimen. Participants receive 12 cycles of induction therapy (Cycle 1 is 21 days; Cycles 2-12 are 28 days). Lenalidomide (Trade name: Revlimid) 20 mg is taken on Days 1-21 of 28-day cycles. Participants achieving a complete or partial metabolic response may receive maintenance SC mosunetuzumab every 8 weeks (Q8W) for an additional 9 cycles.

Route of Administration: Intravenous (IV) or Subcutaneous (SC) for mosunetuzumab; Oral for lenalidomide. **Duration of Treatment:** Up to 12 cycles of induction (approx. 1 year), plus up to 9 cycles of maintenance (approx. 1.5 years).

5.2 Concomitant & Prohibited Therapy Permitted: IV tocilizumab as needed for adverse reactions such as Cytokine Release Syndrome (CRS) defined by protocol. **Prohibited:** Concurrent investigational agents, live attenuated vaccines within 4 weeks before the first dose, or other systemic anti-cancer therapies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Histology Confirmation, Baseline PET-CT
Baseline	Day 0	Treatment assignment, First Dose of Mosunetuzumab (step-up dosing)
Induction Cycles 1-12	Days 1-21/28	Administration of Mosunetuzumab + Lenalidomide, Safety Labs, Efficacy Assessment (PET-CT)
Maintenance	Q8W	Maintenance SC Mosunetuzumab (for responders), Routine Monitoring
End of Study	Follow-up	Final Vital Signs, Exit Interview, IP Accountability, Survival Tracking

7. Outcome Measures (Endpoints)

7.1 Primary Outcomes - Dose-Limiting Toxicities (DLTs) - Percentage of Participants with Adverse Events - Cumulative Area under the Curve over Cycles 1-3 (AUC1-3) of Mosunetuzumab

7.2 Secondary & Exploratory Outcomes - Complete Response Rate (CRR) as determined by the investigator (non-randomized stage) - Complete Response Rate (CRR) as determined by Independent Review Committee (IRC) (randomized stage) - Objective Response Rate (ORR) as determined by the investigator (non-randomized stage) -

Duration of Response (DOR), Progression-Free Survival (PFS), and Overall Survival (OS)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection of Treatment-Emergent Adverse Events (TEAEs), specifically monitoring for Cytokine Release Syndrome (CRS) graded per ASTCT criteria, and neutropenia. **Serious Adverse Events (SAEs):** Routine monitoring and reporting per regulatory guidelines. **Data Monitoring Committee (DMC):** Internal safety monitoring mechanisms managed by the sponsor (Roche).

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy endpoints; Safety Set for all patients receiving at least one dose of the study drug. **Sample Size Justification:** Target N = 237 appropriately powered to evaluate efficacy endpoints and establish the safety/pharmacokinetic profile of SC mosunetuzumab. **Handling of Missing Data:** Handled per protocol standards; non-responders or missing assessments are generally treated as lack of response for categorical endpoints.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring for protocol compliance, source data verification, and AE reporting. **Audit Trail:** Validated eCRF systems to ensure data integrity and track clarification forms.

11. Study Identifiers & Governance

Primary ID: C041942 **Registry Number:** NCT04246086 **Sponsor/Lead Organization:** Roche (Hoffmann-La Roche) **Study Title:** A Study Evaluating the Safety, Pharmacokinetics, and Efficacy of Mosunetuzumab + Lenalidomide (+Len), and the Safety, Tolerability, and Pharmacokinetics of SC Versus IV Mosunetuzumab + Len in Participants With Follicular Lymphoma **Official Regulatory Title:** A Phase Ib/II, Open-Label, Multicenter Study With a Non-Randomized Stage Evaluating the Safety, Pharmacokinetics, and Efficacy of Mosunetuzumab Plus Lenalidomide (+Len), and a Randomized Stage Evaluating the Safety, Tolerability, and Pharmacokinetics of SC Versus IV Mosunetuzumab + Len in Patients With Follicular Lymphoma

12. Clinical Framework (Follicular Lymphoma Specific)

Primary Condition: Follicular Lymphoma (FL) **Diagnosis Threshold:** Histologically documented CD20+ FL (Grade 1, 2, or 3a) **Medical Methodology:** Targeted Immunotherapy + Immunomodulatory Therapy **Optimization Target:** Achievement of Complete Response without the use of traditional chemotherapy

13. Study Procedures & Methodology

Management Pathway: Standard oncology pathway for lymphoma, utilizing Lugano criteria for response assessment. **Education Protocol (HCP):** Site initiation visits and detailed protocol training on managing Cytokine Release Syndrome (CRS) and step-up dosing. **Education Protocol (Patient):** Informed consent processes detailing SC versus IV administration expectations, lenalidomide risk minimization plan, treatment duration, and potential AEs. **Quality Audit Mechanism:** Periodic audits by the Sponsor; Independent Review Committee (IRC) for unbiased endpoint assessment.

14. Observation & Follow-Up Schedule

Total Duration: Up to 12 months induction + 18 months maintenance, followed by long-term survival follow-up. **Onsite Visit Cadence:** Frequent visits during Cycle 1 (Days 1, 8, 15) for step-up dosing, followed by Day 1 of Cycles 2-12, and Q8W during maintenance. **Remote Monitoring Frequency:** As per specific site/sponsor monitoring guidelines. **Checklist Review Interval:** Regular review prior to each cycle for toxicity clearance before dosing.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead	
Statistician	[Name]	_____	[DD-MMM-YYYY]				